



Armada Kalamunda Group

Advance Health Directive and Enduring Power of Guardianship Guideline

1. Purpose

This guideline aims to outline the minimum standard for the Armada Kalamunda Group (AKG) to demonstrate compliance with the requirements of the [Guardianship and Administration Act 1990](#) and the [Guardianship and Administration Regulations 2005](#).

2. Applicability

This guideline applies to Allied Health, Medical, Nursing and Midwifery and HIMS working across all departments within Armada Kalamunda Group.

3. What is an Advance Health Directive?

- An Advance Health Directive (AHD) is a legally binding document that contains a person's decisions about future treatment in anticipation of a time when they may be unable to make reasonable judgments for themselves. Treatment includes medical, surgical and dental treatment and other health care.
- Treatment decisions contained within the AHD can only be enacted if they are clinically appropriate.
- The AHD outlines treatment decisions in which a person consents to, or refuses to consent to, future treatment according to specified circumstances.
- The document DOES NOT include decisions about organ donation. People, who wish to donate their organs and tissues, can register on the [Australian Organ Donor Register \(external site\)](#).

4. What is an Enduring Power of Guardianship?

- An EPG is a legal document in which a person appoints another person to make personal, lifestyle and health care treatment decisions on their behalf in the event that they are unable to make reasonable judgments about these matters in the future.
- Treatment decisions contained within the EPG can only be enacted if they are clinically appropriate.
- This is different from an Enduring Power of Attorney (EPA) whereby a person is appointed to only make decisions on financial and/ or property issues.
- The scope of decisions can be limited by the appointer and should be specified in the EPG form.

5. Further information

- The validity in other states and territories of an AHD or EPG is dependent on the laws applying in that jurisdiction.
- AHDs or EPGs from other states must be validated by the State Administrative Tribunal or re-enacted by the patient in accordance with WA Legislation.
- AHDs or EPGs can be used to direct treatment to be withheld.
- AHDs or EPGs can be uploaded to My Health Record (MHR) by patients. Clinicians are encouraged to check MHR for these documents. Receipt and storage would be treated the same as when a patient provides the document (see [7.2](#)).
- Voluntary Assisted Dying (VAD) cannot be included in an AHD as a treatment decision in respect of a person's future treatment.

Arrangements for VAD: please refer to the EMHS [Voluntary Assisted Dying Management Procedure](#)

6. How is an AHD / EPG made?

- Must be at least 18 years of age and in the case of an appointed EPG, they must be at least 18 years of age.
- Must be made voluntarily and without inducement or coercion.
- Person making AHD / EPG must have capacity to understand the nature and effect of the AHD / EPG.
- Must be prepared by the person themselves (unless the person is not able to write) and witnessed by two people (one of whom must be authorised to witness statutory declarations).
- Where the subject of the AHD / EPG is a patient at AKG, staff (with the exception of a consultant) are not permitted to sign as a witness to either document. This applies also to EPA.
- Must be prepared in Western Australia. AHDs or EPGs from other states must be validated by the State Administrative Tribunal.

For further information on how to make an AHD / EPG refer to the resources available via [Advance Health Directives \(external site\)](#).

7. Guidelines for AHD / EPG and treatment decisions

7.1 AHD Clinical Alert on the Patient Administration System (webPAS):

- When a patient presents to hospital (whether as an emergency presentation or a planned admission) it is important to identify if the patient has an AHD and ensure the presence of the patient's AHD is recorded as an AKG [Adverse Drug Reaction and Clinical Alert Procedure](#) and filed at the front of the patient's medical record.
- The details of the AHD are not recorded in webPAS. They are only recorded on the copy in the Medical Record. If the medical record is off site it can be recalled urgently if required. The alert must be communicated to the clinicians caring for the patient.
- On identifying an AHD alert the Medical Officer must discuss the AHD with the patient or the family/carer to determine the content and currency of the AHD.

For further information regarding the process for recording of an AHD clinical alert on the webPAS please see the AKG [Adverse Drug Reaction and Clinical Alert Procedure](#).

7.2 Receipt and storage of an already prepared AHD / EPG:

The patient provides the health care professional with an already prepared AHD / EPG at any time during their hospital stay:

- Ensure that the patient retains their original document and they are giving you a copy (preferably a certified copy). If they have given you their original, then make a copy and return the original back to them.
- Ensure that the Medical Officer and Nurse/Midwife in charge of the patient's care are made aware.
- The copy of the AHD/EPG should be placed at the front to the patient's medical record behind the Alert divider. Where multiple forms of the same number are present, file in reverse chronological order.
- Alert forms/correspondence without MR numbers are to be filed at the back of the alert divider in reverse chronological order.

The patient forwards an AHD / EPG to the health service by mail:

- The documents are forwarded to HIMS Manager for initiation of the Adverse Drug Reaction and Clinical Alert, then the Medical Records Department for priority filing.
- In the event that the person is not registered on webPAS (does not have a medical record), the AHD / EPG is to be returned to the sender with advice that it is not able to be filed by the health service at that time.

7.3 Patient wishes to prepare an AHD / EPG during the health service episode:

In the event that the patient is to be discharged (and where appropriate) encourage the patient to prepare the AHD / EPG at home and/or in consultation with their General Practitioner (GP) or outpatient practitioner.

In the event that the patient has no prospects for discharge:

- The patient's capacity must be verified, and the capacity assessment is to be documented in the integrated progress notes.
- Refer to the resources available via [Advance Health Directives \(external site\)](#).
- The patient is to be provided with a copy of:
 - [Preparing an Advance Health Directive \(includes form\)](#); or
 - [A Guide to Enduring Power of Guardianship in Western Australia \(includes form\)](#)

7.4 When should an AHD / EPG be used?

- Only to be used when the patient is unable to make a treatment decision in relation to the circumstances specified in the AHD or EPG.
- An AHD comes into effect only if the patient is unable to make reasonable judgements about a treatment decision at the time that the treatment is required. In these circumstances, the AHD acts as the patient's 'voice'.
- The medical practitioner providing the treatment is to ensure that the AHD / EPG has been appropriately completed and witnessed.
- Treatment decisions contained within the AHD can only be enacted if they are clinically appropriate.

8. Consent to Treatment:

When a patient is unable to make reasonable judgments about a treatment decision at the time that the treatment decision is required, who can make the treatment decision on their behalf depends on whether the treatment is “urgent” or “non-urgent”.

8.1 Urgent treatment

Urgent treatment is treatment that is urgently required to:

- save the patient’s life;
- prevent serious damage to the patient’s health; or
- prevent the patient from suffering or continuing to suffer significant pain or distress.

Where urgent treatment is required, the health professional may make the treatment decision unless either:

- it is practical to ascertain whether the patient has made an AHD;
- it is practical to consult with a person who can make the treatment decision on behalf of the patient (this is explained further below as being the Enduring Guardian, Guardian or “person responsible” for the patient).

Even where it is practical to ascertain if there is an AHD or to consult with a person who can make the treatment decision on behalf of the patient, and the patient needs the urgent treatment as a consequence, the health professional can make the treatment decision on behalf of the patient.

8.2 Non-urgent treatment

Non urgent treatment is planned or routine medical, surgical or dental treatment or other health care. In relation to non-urgent treatment, treatment decisions should be made in accordance with the Treatment Hierarchy ([Appendix 2](#)).

For further information regarding consent to treatment refer to the [WA Health OD 0657/16 Consent to Treatment Policy](#).

9. Revocation of an AHD / EPG:

A patient (of capacity) can choose to revoke their AHD / EPG at any time either verbally or in writing; with the preference to be in writing. A copy of the revoked document should be forwarded to the HIMS Manager to ensure the alert is removed from webPAS.

9.1 For the admitted inpatient

The attending Medical Practitioner is to:

- Document ‘REVOKED by (patient’s name) on (date)’ across the front page of the AHD / EPG and sign, date and print their name.
- Advise the patient that the AHD / EPG is no longer valid and will not be used.
- Advise the patient that should they wish to have an AHD / EPG they will need to prepare a new one.
- The revoked document should be retained in the patient’s medical record.

10. Amendments to an AHD / EPG:

- An AHD / EPG cannot be amended.
- In the event that a patient wishes to change their AHD / EPG, the preceding document should be revoked, and the patient advised to prepare a new one.
- Creating a new AHD / EPG will require the patient to comply with the requirements outlined in the [Preparing an Advance Health Directive \(includes form\)](#) or [A Guide to Enduring Power of Guardianship in Western Australia \(includes form\)](#) document.

11. AHD and Goals of Patient Care:

If an AHD indicates a refusal of cardiopulmonary resuscitation (CPR) in the event of cardiorespiratory arrest, it is recommended that a [Goals of Patient Care eForm Procedure](#) is completed via iSOFT, printed and placed into the patient's medical record for each new admission episode, as this is a more recognisable communication tool within the hospital setting.

Where there is conflicting information on the GOPC and AHD, the clinician is to liaise with the patient and/or family members to ensure instructions align and amend as required.

For further information regarding goals of patient care refer to the AKG [Goals of Patient Care Policy](#) and AKG [Goals of Patient Care eForm Procedure](#).

12. Under what circumstances should issues be referred to the State Administrative Tribunal (SAT)?

- Where there is confusion regarding the validity of an AHD / EPG or the appropriate substitute decision maker; the matter should be referred to SAT.
- Where decisions made by the EPG (or substitute decision maker as per the [Treatment Hierarchy](#)) conflict with what the Medical Practitioner considers is likely to be in the patient's best interest; the matter should be referred to SAT.
- Where a patient has been deemed to lack decision making capacity and there is no substitute decision maker as outlined by the Treatment Hierarchy (Appendix 2), or when the substitute decision makers cannot reach a unanimous decision.
- To refer to the SAT the attending Medical Practitioner should contact them via phone 1300 306 017.

13. Legislative context

The following documents are required to give effect to this policy:

- [Guardianship and Administration Act 1990 \(external site\)](#)
- [Civil Liability Act 2002 \(external site\)](#)
- [Criminal Code Act Compilation Act 1913 \(external site\)](#)
- [Mental Health Act 2014 \(external site\)](#)
- [Voluntary Assisted Dying Act 2019 \(External Site\)](#)
- HDWA [Consent to Treatment Policy OD 0657/16](#)
- HDWA [Clinical Handover Policy MP 0095'](#)
- HDWA [Clinical Alert MedAlert Policy MP 0053/17](#)

- Chief Psychiatrist of Western Australia [Reporting Notifiable Incidents – Public Mental Health Services](#)

14. Supporting information

The following documents support the implementation of this policy:

- [EMHS Health Record Access and Disclosure Policy EMHS: 10](#)
- [EMHS Voluntary Assisted Dying Management EMHS: 223](#)
- [EMHS Consent Policy EMHS: 5](#)
- [Goals of Patient Care Policy AKG-POL-0181-18](#)
- [Goals of Patient Care eForm Procedure AKG-PRO-0569/21](#)
- [Adverse Drug Reaction and Clinical Alert Procedure AKG-PRO-0014-15](#)

15. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.6 Sharing decision and planning care
- Standard 5 – Action 5.17 Comprehensive care at the end of life
- Standard 5 – Action 5.19
- Standard 6 – Action 6.7 Clinical handover

16. References

1. Western Australia. Department of Health WA Cancer and Palliative Care Network. (2017). The End-of-Life Framework: A state-wide model for the provision of comprehensive, coordinated care at end-of-life in Western Australia. Retrieved from http://ww2.health.wa.gov.au/Articles/A_E/End-of-life-framework

17. Glossary

The following definitions are relevant to this policy.

Term	Definition
Advance Health Directive (AHD)	Means an advance health directive made under Part 9B of the <i>Guardianship and Administration Act 1990</i>, including an instrument recognised as such under 110ZA of the GA Act. Part 90 applies to treatment decisions in relation to patients under legal incapacity and specifies that ‘advanced health care directives’ include common law directives, in relation to a person under legal incapacity. Therefore, AHDs include: • those which comply, or substantially comply with the form prescribed under the GA Act • those made under equivalent provisions in other Australian jurisdictions which have been recognised as AHD by an order made by the State Administrative Tribunal

Term	Definition
	common law directives, in relation to a person under legal incapacity.
Capacity	In the context of medical treatment, a patient has capacity if he/she is capable of understanding the nature, purpose and consequences of the proposed treatment. Capacity must always be assessed in the context of the decision that is to be made. The Mental Health Act 2014 (s15) defines a person as having capacity when they: - understand any information or advice about the decision that is required - understand the matters involved in the decision - understand the effect of the decision - weigh up the above factors for the purpose of making the treatment decision - communicate the decision in some way.
Carer	A person who (without being paid) provides ongoing care or assistance to another person who has a disability, a chronic illness, including a mental illness or a person because of frailty requires assistance with carrying out everyday tasks.
Enduring Power of Guardianship (EPG)	A formal document in which a person nominates a competent adult to be an Enduring Guardian (see the <i>Guardianship and Administration Act 1990</i> Part 9A).
Guardian	An adult appointed by the State Administrative Tribunal to make wide-ranging decisions, including treatment decisions, on behalf of an adult person who does not have the capacity to make such decisions for him/herself (see the <i>Guardianship and Administration Act 1990</i> Part 5).
State Administrative Tribunal (SAT)	Independent tribunal established under the State Administrative Tribunal Act 2004 to hear disputes and make determinations about a range of administrative matters. Examples of proceedings relevant to health include health practitioners' disciplinary matters and guardianship matters.

18. Document owner

Director Clinical Services

Enquiries relating to this guideline may be directed to:

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19. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2.1	10/08/2022	10/08/2025	Updated hyperlinks – change in legislation

20. Authorisation

This guideline has been authorised and issued by the AKG Executive Director.

Authorised by	Dr Alison Parr – Director Clinical Services
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Appendix 1: Treatment Hierarchy

Extracted from the [WA Health Consent to Treatment Policy](#).

